

Emergency Psychiatry

Paper B

Syllabic content 7.4

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1. Emergency Psychiatry: Introduction

Goals of emergency psychiatric care

- A. Triage: Collect first line information, prioritise as per need and communicate/escalate appropriately to arrange further care
- B. Assessment: Appropriate assessment to seek further diagnostic information pertinent to manage the presenting problem. A rapid understanding of aetiology is a key skill required in this context.
- C. Diagnosis: Accurate diagnostic workup is necessary for formulation and management
- D. Short term management: This involves options such as medication use, hospitalisation, seclusion/restraint and crisis social interventions and psychoeducation.
- E. Discharge planning: Longer term interventions should be planned including preventative strategies aimed at averting crisis and rational follow-up strategies.

Common emergencies

General syndromes	Iatrogenic issues
•Suicidality •Agitation •Confusion •Starvation •Catatonia •Disinhibition •Intoxication / Withdrawal syndromes	•Acute dystonia •Neuroleptic malignant syndrome •Serotonin syndrome •Lithium toxicity •Clozapine-related agranulocytosis

Differential diagnoses

Agitated patient	
Acute confusion, known physical frailty, associated signs of metabolic/electrolyte disturbances or infections	Medical/Neurological syndromes
Disorientation, impaired consciousness, ataxia, autonomic dysfunction, hallucinations	Intoxication/Withdrawal of alcohol and/or other substances
Disorganised speech, suspiciousness, delusions, hallucinations	Acute psychotic disturbance
Memory disturbance, Disorientation, Disinhibition, other cognitive difficulties	Dementia
Past history of forensic contacts, premeditated behaviours, verbal threats, targeting victims	Antisocial personality
Increased drive, Flight of ideas, elevated or irritated mood	Mania
Subdued mood, anxiety Catastrophic/guilt delusions	Agitated depression
Intense fear, shortness of breath, palpitations, autonomic manifestations	Panic attack, Generalized anxiety
Subjective restlessness, motor unrest predominantly in the legs.	Akathisia due to neuroleptic use

Catatonic patient		
No previous history of psychiatric disorders, worsening with benzodiazepines	→	Organic causes e.g. encephalitis, tumour
Low mood, past history of depression, hopelessness	→	Depressive stupor
Hallucinations, delusions, paranoia, disorganised behaviour	→	Schizophrenia
Pressured speech, grandiosity, loss of sleep	→	Manic excitement
Rapid onset, marked rigidity, autonomic instability without posturing	→	Psychotropic induced e.g. NMS
Impaired intelligence, other stereotypic behavior patterns.	→	Autism / Neurodevelopmental disorders

Starving patient		
Self-inflicted, body image distortion, preceding weight reduction attempts	→	Eating disorders esp. anorexia nervosa
Low mood, past history of depression, hopelessness	→	Psychotic depression
Hallucinations, delusions, suspicions about poisoning, disorganised behaviour	→	Schizophrenia
Pressured speech, grandiosity, loss of sleep, risk indulgence	→	Manic neglect
Rapid onset, marked rigidity, autonomic instability without posturing	→	Psychotropic induced e.g. NMS
Recurrent irrational worries about safety, ritualistic behaviour	→	OCD with food-related obsessions

2. Suicidal patients

Epidemiology

Suicide, defined as completed death from an intentional self-initiated act, accounts for 1 to 2% of total global mortality (1 in 6000/year). It is the fifth most common cause attributed to YLL - years of life lost in the developed world. It accounts for 1% of deaths in England and Wales: Total = 8 per 100 000 per year.

Suicide rates show global fluctuation that follow macro-level socioeconomic conditions e.g. absolute suicide rates dropped during times of World Wars. In general the rates are declining in most groups but increasing among young men.

- ★ **Men:** Hanging was the most commonly used method for suicide by men accounting for almost 40 per cent of all deaths followed by drug overdose (20 per cent) and self-poisoning by car exhaust fumes (almost 10 per cent), drowning and jumping.
- ★ **Women:** The commonest methods for women were drug overdose (46 per cent) hanging (almost 27 per cent) and drowning (7 per cent) (Brock and Griffiths, 2003).
- ★ **Older people:** In most countries the highest rate of suicide is among people aged over 75 years. The most frequent methods are hanging among men, and drug overdose among elderly women (Harwood et al., 2000a). In addition to active self-harm, some older adults die from deliberate self-neglect, for example by refusing food or necessary treatment. As in younger age groups, depression is a strong predictor of suicide in the elderly. Other predictors are social isolation and impaired physical health though the latter may act in part through causing depression (Conwell et al., 2002). Personality traits are also important risk factors especially anxious and obsessional traits (Hardwood et al., 2001b).
- ★ **Children and adolescents:** Suicide is rare in children, though the rates have shown an alarming increase among adolescents in recent years. In England and Wales the increase has been mainly in males aged 15-19 years (McClure, 2000) and principal methods among males have been hanging and poisoning with car exhaust fumes (Hawton et al., 1999a). Children who die by suicide have usually shown antisocial behavior & suicide behavior and depressive disorders are common among their parents and siblings (Shaffer, 1974).
- ★ **Ethnic differences:** In the UK there is particular concern about disproportionately higher rates of suicide amongst Asian women.
- ★ **High-risk occupational groups:** The suicide rate among doctors is greater than that in the general population and the excess is greater among female than male doctors (Hawton et al., 2000). Anesthetists, general practitioners and psychiatrists are at particularly higher risk (Hawton et al., 2000). Farmers also have higher rates of suicide.

Possible causes include the ready availability of means of self-harm (such as poisons and guns) together with stress related to work and financial difficulties (Malmberg et.al.1999).

Rate of mental disorders

Frequency of mental disorders from psychological autopsy studies on completed suicides (Isometsa et al.,2001)

Depressive disorders	36-90%
Alcohol dependence or abuse	43-54%
Drug dependence or abuse	4-45%
Schizophrenic disorder	3-10%
Organic mental disorder	2-7%
Personality disorders	5-44%

- ★ **Depression and suicide:** About 6-10% of those who suffer from a mood disorder will die by suicide. The risk is highest for depressed inpatients and lowest for those treated for depression in primary care. History of impulsive and aggressive behavior, alcohol and drug abuse and dependence, and cluster B personality disorders increase the risk of suicide in individuals with major depression. The greatest risk of suicide is seen in those with a history of attempted suicide, wherein the risk increases by a factor 40 times higher.
- ★ **Alcohol and drugs,** often combined, are a major risk or a precipitating factor for suicide. The lifetime risk of suicide has been estimated at 7% for alcohol dependence, with only slight variation over the life. The suicide rate in heavy drinkers is 3.5 times higher and in those with diagnosable alcohol use disorders is 10 times higher than in the general population. In drug dependence or abuse it is 15 times higher than expected.
- ★ In **anorexia nervosa** the risk is about 20-fold higher than general population.
- ★ The lifetime risk of suicide in **schizophrenia** is estimated to be 6%, almost 10 times higher than in the general population. The great majority of schizophrenic patients commit suicide in the active phase of the disorder after having suffered depressive symptoms.
- ★ **Personality disorders** are strongly associated with suicide. Most suicide victims with a personality disorder have comorbid depressive disorder or substance abuse. They often suffer from impulsive and aggressive behavior. Rates of personality disorders are higher in younger victims.

The table below has been compiled using data from **National Confidential Enquiry into Suicide and Homicide in the UK**.

Description	Rates
1. Global annual suicide rate	1 in 6000/year
2. Male:female	2-4:1
3. Most common age	• -15 to 24 females; 25 – 34 males. • > 65 declining, 15 – 24 increasing
4. Commonest methods	Hanging, overdose
5. The most common psychiatric diagnoses in suicide	Major depression (30–31%) & alcohol dependence (17–24%).
6. Mental disorders without much increase in suicide rates	Mental retardation and dementia. OCD lower rates than others – BUT ONLY IF NO CLINICAL DEPRESSION.
7. Suicides that have at least one recorded DSH attempt	40 - 60%
8. Number that will repeat DSH within one year	30%
9. In contact with mental health services within a year of death	25%
10. Those on psychiatry outpatient registers	25%
11. Attempted suicides under alcohol influence	25% of all attempted suicides; 50% of has had alcohol within the previous 6 hours.
12. Having seen by psychiatrist in previous week	12.5%
13. Having seen health worker in last 3 weeks	33%
14. Having seen GP in last four weeks	66%
15. Having seen GP in last one week	40%
16. Inpatient suicide	Hanging, belt, curtain rail
17. Inpatient suicide in first week of admission	25% of all inpatient suicides
18. Inpatient suicides when under routine (not constant or intermittent observations)	80% of all inpatient suicides
19. Noncompliant with medications	20%
20. Within 3 months of discharge from ward	25% suicides; 40% of these occurred before first follow up. In first 28 days after discharge, 1 in 500-1000 patients commit suicide.
21. Preventable suicides according to mental health teams in England	22% (especially inpatient suicides)

22. Strongest risk factor of suicide	DSH history (attempted suicide)
23. Risk of suicide within one year of DSH	0.7% (nearly 1 in 100); more in males 1.1%; 0.5% in females. This is 66 x the general population risk.
24. Enhanced Care Programme Approach cases	Nearly 50%
25. Older patients who committed suicide that visited their primary care physician	20% on the same day as their suicide, 40% within 1 week, and 70% within 1 month

Mental health teams in England and Wales regard 22% of the suicides as preventable and in around 67% of recorded suicides identifiable factors that could have reduced risk were noted (mainly improved patient compliance and closer supervision).

- **Inpatient suicides:** Around one-third of inpatient suicides involved patients that were on agreed leave at the time of death.
- **Post discharge suicides:** These are associated with inpatient admissions lasting less than 7 days, a discharge from a previous admission having occurred within 3 months of final admission and increased rates of self-discharge. 40% post discharge suicides in England and Wales, 35% in Scotland, and 66% in Northern Ireland, occurred before the first follow-up appointment.
- **Missed contact:** Nearly 30% of suicides in the community missed their most recent appointment with services.
- **Ethnic minorities:** Suicides in ethnic minorities are usually associated with more severe mental illness; 75% of Black Caribbean patients who commit suicide in England & Wales have a diagnosis of schizophrenia. Suicides in ethnic minorities are also associated with higher rates of recent onset treatment non-compliance.
- **Homelessness:** 3% of suicides in England and Wales, 2% in Scotland, and 1% in Northern Ireland involved homeless subjects. 71% of suicides among homeless occurred during or immediately after inpatient care.

Nonfatal self-harm

- **Suicidal ideas:** (Nock et al, *The British Journal of Psychiatry* (2008) 192: 98-105) According to 17 countries data as a part of WMH survey initiative, the cross-national lifetime prevalence of suicidal ideation, plans, and attempts is 9.2%, 3.1% and 2.7%. Across all countries, 60% of transitions from ideation to plan and attempt occur within the first year after ideation onset. Non-fatal deliberate self-harm (parasuicide, attempted suicide) refers to intentional self-poisoning /injury without fatal outcome.
- **Suicidal attempts:** Males commit more suicides though females attempt more. Approximately 25 attempts of suicide are recorded for each recorded suicide. Self-poisoning, mostly using prescribed drugs, accounts for 90% of cases. The most

commonly used drugs are the non-opiate analgesics such as paracetamol and aspirin. Antidepressants, both tricyclics and SSRIs are used in ~25% of episodes.

Comparison of completed suicide with deliberate self-harm

Features	Completed suicide	Non-fatal deliberate self-harm
Sex	More males	More females
Age	Late middle age	Late teens/early 20s
Marital status	Widowed>Divorced>Single>Married	No consistent patterns
Social class	No class distinctions. Higher among the unemployed and retired	Lower and unemployed
Early childhood	Death of a parent	Broken home
Family history	Depression, suicide, alcoholism	Similar episodes
Physical health	Handicapped/terminal illness	-
Personality	Various traits esp. cluster B	Cluster B traits predominate
Diagnosis	Major psychiatric disorders and/or substance use	'Mental distress' 'Reactive' depression
Cognitive precipitants	Guilt & hopelessness	Identity difficulties, emotional distress
Setting	Concealed settings	Others are usually present

(Adapted from Puri & Treasden, Emergencies in Psychiatry Pg 117)

- Current estimates of the rate of deliberate self-harm in Britain suggest a figure of about 3 per 1000 per year. Of these, 20% repeat the act in the next year and 1% die within the next year. Overall, 10% eventually complete suicide
- ~33% males and ~25% females consume alcohol in the 6 hours before the act (Hawton et al., 2003b).
- Deliberate self-injury accounts for ~10% of all deliberate self-harm presenting to general hospitals in Britain. The commonest method of self-injury is laceration usually of the forearm or wrists.
- Psychiatric disorders can be detected in ~90% of deliberate self-harm patients seen in hospital settings, if standardized assessments are used (Hawton et al 2001; Suominen et al., 1996). The most frequent diagnosis is depressive disorder followed by dependency or harmful use of alcohol and drugs in men, and anxiety disorders in women. Comorbidity with personality disorders is very frequent.

Factors associated with risk of repeating attempted suicide/DSH
Age 25-54 years
Alcohol or substance use
Diagnosis of personality disorder
History of violence
Lower social class
Previous psychiatric treatment
Previous self harm
Single, divorced, or separated
Unemployment

From Hawton, K (2000a). Treatment of suicide attempters and prevention of suicide and attempted suicide. In MG Gelder, JJ Lopez Ibor Jr, and NC Andreasen (eds). The New Oxford Textbook of Psychiatry, chapter 4.15.4 Oxford University Press.

Risk Assessment

Clinical indicators of high suicidal intent

- ★ Act carried out in isolation and timed so that intervention unlikely
- ★ Precautions taken to avoid discovery
- ★ Precaution made in anticipation of death (e.g. Making will organizing insurance, suicidal note)
- ★ Premeditated actions leading to the final act (e.g. purchasing means, saving up tablets)
- ★ Communicating intent to others beforehand
- ★ Not altering potential helpers after the act
- ★ Admission of suicidal intent
- ★ Ongoing plans to repeat the act
- ★ Continued access to means/tools to reattempt suicide
- ★ Dignosable mental illness (e.g. depression, bipolar disorder, schizophrenia) or personality disorder (e.g. borderline personality disorder)
- ★ Physical illness, especially chronic conditions and / or those associated with pain and functional impairment (e.g. epilepsy, multiple sclerosis, malignancy, pain syndromes)
- ★ Recent contact with psychiatric services
- ★ Recent discharge from psychiatric inpatient facility.

Demographic indicators

- ★ Male
- ★ Low socio-economic status
- ★ Unmarried, separated, widowed
- ★ Unemployed and living alone.

Risk factors for completing suicide	
Past self harm	Psychiatric history
Older age	Unemployment
Male	Poor physical health
Social isolation	Access to means

Background history

- ★ Deliberative self-harm (especially with high suicide intent)
- ★ Childhood adversity (e.g. sexual abuse)
- ★ Family history of suicide
- ★ Family history of mental illness
- ★ Clinical history

Psychological and psychosocial factors

- ★ Hopelessness
- ★ Impulsiveness
- ★ Low self-esteem

- ★ Recent stressful life event
- ★ Relationship instability
- ★ Lack of social support.

Current 'context'

- ★ Suicidal ideation
- ★ Suicide plans
- ★ Availability of means

Various structured tools are available for risk assessment.

SAD PERSONS Score: 10 major demographic risk factors used in a mnemonic to assess immediate suicidal risk often in acute general hospital setting. The scores can guide in making a decision to admit or discharge a patient.

S – Sex: 1 if male; 0 if female; (more females attempt, more males succeed)
A – Age: 1 if < 20 or > 44
D – Depression: 1 if depression is present
P – Previous attempt: 1 if present
E –Ethanol abuse: 1 if present
R – Rational thinking loss: 1 if present
S – Social Supports Lacking: 1 if present
O – Organized Plan: 1 if plan is made and lethal
N – No Spouse: 1 if divorced, widowed, separated, or single
S – Sickness: 1 if chronic, debilitating, and severe

Beck Hopelessness Scale consists of 20 true-false statements focused on pessimism and negativity about the future. The degree of hopelessness measured using this tool is a good indicator of suicidal risk with scores: 0 –3 indicating minimal, 4 – 8 mild, 9 –14 moderate, and 15–20 severe risk.

Beck Scale for Suicidal Ideation is a self-report 24-item scale (5 screening items) that assesses a patient's thoughts, plans and intent to commit suicide. The total scores could range from 0 to 48 (each item scored from 0 to 2). Higher scores reflect greater suicide risk, though no defined cut-offs are identified for categorizing the risk profiles.

Risk management plan

There are four important questions that might help in decision making and formulating a safe management plan, following the assessment

- Is there evidence of mental illness?
- Is there an on-going suicidal intent?

- c. Are there non-mental health issues that can be addressed?
- d. What is the level of social support available at present?

Having assessed the suicidal risk, clinician should make a treatment plan and decide whether the patient should be admitted to hospital or treated as an outpatient. This decision depends on the intensity of the suicidal intention, the severity of any associated psychiatric illness and the availability of social support outside hospital. (Adapted from Gelder et al.(eds), Shorter Oxford Textbook of Psychiatry, 3e. pg 415)

Care of the potentially suicidal patient in the community
Full mental health and social context assessment
Considering place of safety
Regular review of the suicide risk and the arrangements
Safe treatment using adequate dosage of preferably less toxic drugs
Restricting prescriptions and other means of access
Involving relatives for the safe keeping of tablets
Arrangement for crisis or urgent access for the patient and carers

Adolescent suicides

Suicidal ideation (without deliberate self harm) in the past year was reported by 15.0% of an adolescent cohort in UK (school pupils – self report). This was more common in females (22%) than males (8.5%) (Odds ratio 3.1).

FACT	FIGURE
Most common methods: Paracetamol overdose and cutting	
One year prevalence of self-harm among 5-10 year-olds without any mental health issues	= 0.8%
One year prevalence of self-harm among 5-10 year-olds diagnosed with an anxiety disorder	= 6.2%
One year prevalence of self-harm among 5-10 year-olds if the child had a conduct, hyperkinetic or less common mental disorders	= 7.5%
One year prevalence of self-harm among 11-15 year-olds without any mental health issues	= 1.2%
One year prevalence of self-harm among 11-15 year-olds diagnosed with an anxiety disorder	= 9.4%
One year prevalence of self-harm among 11-15year-olds if the child had a conduct, hyperkinetic or less common mental disorders	= 8-13%
One year prevalence of self-harm among 11-15year-olds if the child had depression	= 18.8%
Proportion of DSH that receives hospital attention	Less than 13%
One year prevalence of self harm in 15-16 year olds	= 6.9%
Of all adolescents - Proportion of under16 group in A&E attendees with self harm	= 5%
Proportion that self harm at least once a week	= 41%
Proportion that self harm at least once a week	= 27%
There is no difference in prevalence between adolescents from the white or black or ethnic minority communities.	

3. Managing Other Emergencies

Managing Agitation

Agitation is a broad term; it may or may not be associated with a threat of imminent violence. Certain risk factors for violent crime are tabulated below.

Factors associated with dangerousness
Age (younger age)
Males predominate
Past history of criminality and violence
Experience of physical or sexual abuse as a child
Childhood conduct disturbances
Psychiatric diagnosis (e.g. mania, schizophrenia, alcoholism or other substance dependence, conduct disorder, antisocial personality disorder, and intermittent explosive disorder)
Conducive environment
Specific symptoms (especially command hallucinations, agitation, and hostile suspiciousness)
Unemployment

Management of an extremely agitated patient: The initial steps should be securing safety of the patient and others in the environment, verbal de-escalation and defusing. Pharmacological options must be considered as second line interventions.

Lorazepam (1–2 mg intramuscularly or orally) or haloperidol (5 mg orally or intramuscularly) can be used for agitation. Olanzapine IM 5-10mg is also approved for this purpose. More recently, promethazine 25-50mg PO repeated after 1-2 hours, max. 100mg has been included in the NICE recommendations for rapid tranquilisation.

Side effects	Immediate actions
Bradycardia or irregular pulse (<50pm)	Seek urgent physician input
Acute dystonia	Use procyclidine 5-10mg IM or benztropine 1-2mg
Reduced breathing rate (<10pm), falling O2 saturation	Use flumazenil if previous use of benzodiazepines noted; if other agents used arrange ITU transfer for mechanical ventilation
Drop in BP (orthostatic drop, <50mm Hg diastolic)	Lie flat; raise legs, refer to physicians if no response

Managing Catatonia

Two subtypes of catatonia can present with different challenges. In withdrawn catatonia, self neglect leading to starvation and dehydration needs to be addressed. In agitated/excited variant, dangerousness needs to be managed. Untreated catatonia may also lead to physical complications such as electrolyte imbalance and renal damage.

Apart from depression, mania and schizophrenia, several neurological conditions (e.g. postencephalitic states, parkinsonism, seizures, basal ganglia damage, stroke and tumours) may be associated with catatonia. General medical conditions causing delirium can also present with catatonic features. In addition NMS can also present as catatonia.

In most catatonic patients, inpatient care will be required. The use of lorazepam (1–2 mg, intramuscularly or intravenously) is recommended, in addition to the treatment of underlying psychiatric syndromes.

NMS, if suspected, should be treated as a medical emergency. The offending neuroleptic must be stopped immediately and supportive treatment should be instituted (rehydration, using antipyretics). Also consider using dantrolene (2–3 mg/kg) or bromocriptine (2.5–10 mg three times daily) after transferring to medical ITU.

Managing Confusion

In a confused or delirious patient, optimising the patient's environment is the most important immediate intervention. This can be done by promoting the presence of familiar people in the vicinity, using well-lit nursing bay, using frequent reorientation to place and purpose, providing one-to-one nursing, encouraging adequate fluid and food intake, avoiding polypharmacy including routine sedatives and other agents that can worsen confusion e.g. anticholinergics.

Medication can be used if the patient is agitated or distressed. In the elderly, use haloperidol at 0.5mg, and repeat if necessary after an interval of at least 2 hours. For adults oral haloperidol can be started at 2mg. Treating the underlying cause is the most appropriate long term solution.

Managing Self-Neglect/Starvation

A detailed assessment of risk is the first step. In the elderly this assessment must include

- (a) Activities of daily living;
- (b) Environmental assessment
- (c) Cognitive assessment (4)

Detailed physical assessment.

It is also important to assess formally and document the capacity to make treatment decisions in all cases of self-neglect.

Factors associated with self neglect in the elderly
Advanced age
Lack of social contacts
Presence of medical morbidity
Diagnosis of dementia, depression and/or alcoholism
Poverty and illiteracy
Suspicious, mistrustful, avoidant or paranoid personality
Loss of a caregiver
Bereavement and/or other loss events
Decreased vision/hearing or other physical impairments

In cases of anorexia, lab investigations must be done to check for metabolic, endocrine, acid-base and electrolyte complications.

First line management involves restoring nutrition and hydration. This may involve oral replenishment or parenteral administration depending on general medical status. Specific treatment depends on the underlying cause. In psychotic depression, severe self-neglect and/or catatonia warrants the use of ECT. In patients with anorexia, nasogastric feeding may be required.

4. Legislative Aspects

- ★ A patient who has harmed himself but is alert and conscious should be presumed the mental capacity to refuse medical advice unless shown to be otherwise as soon as possible. If appropriate, assessment for compulsory treatment should be arranged.
- ★ When a patient is assessed as mentally incapable (for example, because of persistent intoxicating effects of overdose) staff have a responsibility to act in the person's best interests. This may include taking the person to hospital, keeping them there for assessment and giving immediate life-saving treatment.
- ★ It is important to acknowledge that mental capacity may change over time, and attempts should be made to explain each new procedure or treatment and to obtain consent before it is carried out.
- ★ When, after full discussion a competent patient continues to refuse to consent and there are no grounds for compulsory treatment a further attempt should be made to find an acceptable alternative plan. If the attempt does not succeed the consequences of the decision should be explained clearly, and the discussions recorded fully in the notes.
- ★ If the patient insists on leaving he has to be allowed to go, but should be encouraged to return, and given an emergency contact number and options for further treatment. The situation should be discussed as soon as possible with the general practitioner.

Compulsory measures:

The section below is a brief outline of legal aspects of emergency care in England & Wales. The sections are mentioned only as guidance. We do not expect the specifics of Mental Health Act to be tested in the exams, but the application of legal principles in the context of emergency care may be tested.

The main procedures allowing compulsory detention in hospital are

- ★ Section 2 (**admission for assessment**)- an application for detention under section 2 may be made by the nearest relative or ASW (approved social worker) and requires two medical recommendations one of which must be by an approved doctor. Duration of detention is 28 days. Following the section 2 an application may be made for detention under section 3. Alternatively the patient may remain in hospital informally or be discharged.
- ★ Section 3 (**admission for treatment**)- an application for detention under section 3 is made in a similar manner to section 2. Duration of detention is initially 6 months, which may be renewed for a further 6 months, and then 12 months thereafter.

- ★ Section 4 (**emergency admission**)- It allows the emergency detention of patients who have not yet been admitted to hospital (this includes those in accident and emergency, outpatients, and day hospitals)
- ★ Section 5(2) (**emergency detention**)- It applies to patients who have already been admitted to a hospital (psychiatric or non-psychiatric ward) on voluntary basis. Section 5(2) can be used where a doctor thinks as assessment under the Mental Health Act ought to be undertaken with a view to detention under section 2 or section 3 of the Mental Health Act 1983. It only applies to inpatients. The duration of detention is 72 hours during which an assessment must be undertaken to determine if detention under section 2 or 3 is warranted.

Mental Health Act in a non-psychiatric unit

- ★ It is useful to be aware of the provisions of section 5(2) for urgent detention of a voluntary inpatient, and section 5(4), a nurse's holding of a voluntary inpatient.
- ★ Section 5(2) must be undertaken by the registered medical practitioner in charge of treatment (the consultant in charge of the patient's care or a deputy, e.g. an on-call doctor nominated by him).
- ★ Section 5(2) cannot be used in accident and emergency departments or with outpatients.
- ★ Whilst under section 5 (2) the patient cannot be transferred to another ward as they are technically in a 'place of safety' unless the patient's life is at risk and there would be irreversible serious harm done.
- ★ Section 5(4) allows nurses (of the prescribed class) to hold an informal inpatient in hospital for up to 6 hours to allow for a medical assessment.
- ★ Section 5 (2) does not allow treatment to be given in itself, although this can be done under common law or the Mental Capacity Act 2005.

Mental Capacity Act [MCA]

- ★ Prior to the enforcement of the Mental Capacity Act 2005 patients in England and Wales lacking capacity were treated under common law under (a doctor treats an incapable patient in the patient's best interest as the doctor has a 'duty of care' (doctrine of necessity)).
- ★ The Mental Capacity Act has five main principles:
- ★ Any person is assumed to have capacity unless it is established otherwise.
- ★ An individual should not be regarded as unable to make a decision unless all practical steps to help him do so have been undertaken without success.
- ★ An individual is not to be treated as unable to make a decision merely because he makes an unwise decision.
- ★ Any action or decision made under the Act for or on behalf of an individual who lacks capacity must be done in his best interests.

- ★ Regard must be taken before an act is done or a decision made under the Act as to whether the purpose can be effectively achieved in a less restrictive way in terms of the patient's rights and freedom of action.

Assessment of capacity

- ★ This is a two-stage process:
 - Is there impairment or disturbance in the functioning of the person's mind or brain?
 - If there is does this make the person unable to make a particular decision?

MCA states that the answers to these questions should be decided on a balance of probabilities.

- ★ The following are considered central to the assessment:
 - Understanding information relevant to the decision.
 - Retaining that information
 - Ability to use or weigh up that information as part of the process of making the decision.
 - Ability to communicate the decision, which can include means other than talking such as sign language or writing.

The person carrying out assessments only has to have a reasonable belief about what is in the person's best interests at the end of checklist above.

5. Crisis Resolution & Home Treatment (CRHT) Teams

A CRHT is a team of professionals responding to psychiatric emergencies quickly often at the patient's home. The focus for crisis resolution teams has been to reduce the need for acute psychiatric hospital admission and occupancy of beds. The main reasons for this 'home-treatment' focus are

1. Interpersonal problems are the major causes of psychiatric crises. These are best observed and treated at the settings where they arise. Further, the coping skills learnt when dealing with a psychological crisis are most effectively applied in the context in which they have been learnt.
2. Most patients prefer homecare to hospitals, as the perceived stigma is less at home settings. In particular, the relationships between patients and professionals are different and less dominated by inequalities of power when crises are managed in the patients' own homes. This notion originates from the ideas of deinstitutionalization movement.
3. Home treatment often costs less than inpatient treatment
4. Hospital admission has harmful as well as therapeutic effects.

Crisis resolution teams intervene over a shorter period and with a much wider range of diagnoses. They have been used to reduce the length of inpatient stay by facilitating early discharge with intensive home treatment and support, though in recent times, questions have been raised about their effectiveness within the NHS.

Key characteristics of crisis resolution teams

- ★ Multidisciplinary in nature
- ★ Community based operation
- ★ Rapidly responsive (same day)
- ★ Partnership-working with other mental health teams to ensure continuity of care
- ★ Focused on those emergency situations where admission would otherwise be indicated.
- ★ Ability to maintain contact and facilitate early discharge when a patient is admitted.
- ★ Lower patient-to-staff ratios, with a capacity to visit even up to several times a day, with 24-hr availability, and response within 1-hr when possible. There may be direct administration of medication up to 4 times daily if required.
- ★ Review patient's progress at least daily.
- ★ Have a gate-keeping role, so that no individuals are admitted to an acute psychiatric inpatient unit without the crisis resolution team assessing the patient first and considering whether intensive home support and treatment would avoid hospital admission.

- ★ Usually patients are only under the care of crisis resolution teams for a short period only i.e. less than 2 months.
- ★ Interventions follow standard psychiatric practice with a comprehensive initial assessment followed by standard medication and psychosocial interventions.

DISCLAIMER: This material is developed from various revision notes assembled while preparing for MRCPsych exams. The content is periodically updated with excerpts from various published sources including peer-reviewed journals, websites, patient information leaflets and books. These sources are cited and acknowledged wherever possible; due to the structure of this material, acknowledgements have not been possible for every passage/fact that is common knowledge in psychiatry. We do not check the accuracy of drug-related information using external sources; no part of these notes should be used as prescribing information

References and further reading

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